

CARDIOVASCULAR DECK

THE ESSENTIAL 53 CARDS

Double-Sided Study Cards · Color-Coded Categories

CLINICAL STUDY DECK SETUP

PRINTING SETTINGS

- Paper: Heavy Cardstock (8.5x11")
- Scale: 100% or 'Actual Size'
- Sides: Print Double-Sided
- Binding: 'Flip on Long Edge'
- Color: Print in Full Color
- Test first: print pages 3-4 on plain paper to check alignment

STUDY & ASSEMBLY

- Cutting: Use the dashed grid lines and scissor marks to cut cards precisely.
- Active Recall: Look at the back category color first. Try to recall the details before flipping.
- Notes Box: Use the dashed box on the front to write your own personal memory tricks.

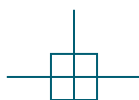
COLOR CODING SYSTEM

Every group below has its own color, shown on both sides of each card. Use the color to group related cards and quiz yourself by class before flipping.

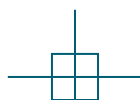
 Beta Blocker (Selective)	 PCSK9 Inhibitor	 Class III Antiarrhythmic
 Beta Blocker (Non-selective)	 Loop Diuretic	 Class IC Antiarrhythmic
 Beta/Alpha Blocker	 Thiazide Diuretic	 Cardiac Glycoside
 ACE Inhibitor	 Thiazide-like Diuretic	 ARNI
 Angiotensin II Receptor Blocker (ARB)	 Potassium-Sparing Diuretic	 Anticoagulant (Vitamin K Antagonist)
 Calcium Channel Blocker (DHP)	 Carbonic Anhydrase Inhibitor	 Anticoagulant (Indirect Thrombin Inhibitor)
 Calcium Channel Blocker (Non-DHP)	 Osmotic Diuretic	 Low Molecular Weight Heparin (LMWH)
 Statin / Antilipemic	 Alpha-2 Agonist	 DOAC / Factor Xa Inhibitor
 Cholesterol Absorption Inhibitor	 Alpha-1 Blocker	 Direct Thrombin Inhibitor
 Bile Acid Sequestrant	 Direct Vasodilator	 Antiplatelet Agent
 Fibrate	 Nitrate / Vasodilator	 Thrombolytic

ALIGNMENT TEST • FRONT

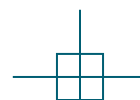
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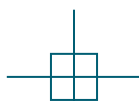
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2



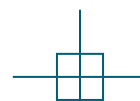
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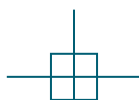
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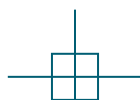
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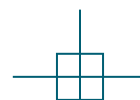
6



7

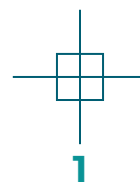
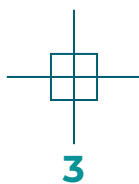


8



9

ALIGNMENT TEST • BACK



METOPROLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Selective)

USE: HTN, angina, heart failure, MI

MOA: Blocks beta-1 receptors, decreasing HR and contractility

SE: Bradycardia, hypotension, fatigue, masks hypoglycemia

NOTES: Check HR/BP before giving. Do not stop abruptly.

My Notes / Clues:

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ATENOLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Selective)

USE: HTN, Angina, Acute MI

MOA: Selectively blocks beta-1 adrenergic receptors

SE: Fatigue, bradycardia, dizziness

NOTES: Hold if HR < 60 bpm. Taper when discontinuing.

My Notes / Clues:

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PROPRANOLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Non-selective)

USE: HTN, migraines, performance anxiety

MOA: Blocks Beta-1 (heart) and Beta-2 (lungs) receptors

SE: Bradycardia, bronchospasm, cold extremities

NOTES: Strictly contraindicated in asthma/COPD patients.

My Notes / Clues:

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CARVEDILOL

CLASS: Beta/Alpha Blocker

USE: Heart failure, HTN

MOA: Non-selective beta and alpha-1 blockade (causes vasodilation)

SE: Bradycardia, orthostatic hypotension, fatigue

NOTES: Take with food to minimize orthostatic hypotension.

My Notes / Clues:

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LABETALOL

CLASS: Beta/Alpha Blocker

USE: Severe HTN, hypertensive crisis

MOA: Non-selective beta and alpha-1 blockade

SE: Orthostatic hypotension, dizziness, scalp tingling

NOTES: Keep patient supine during IV administration.

My Notes / Clues:

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ESMOLOL

CLASS: Beta Blocker (Selective)

USE: SVT, Atrial Fibrillation (Rapid rate control)

MOA: Ultra-short-acting beta-1 blocker

SE: Profound hypotension, diaphoresis

NOTES: Given via continuous IV infusion. Very rapid onset/offset.

My Notes / Clues:

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LISINAPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, heart failure, post-MI

MOA: Blocks conversion of Angiotensin I to Angiotensin II

SE: Dry cough, hyperkalemia, angioedema

NOTES: Monitor BP and K⁺. Teratogenic (avoid in pregnancy).

My Notes / Clues:

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ENALAPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, Heart Failure

MOA: Inhibits ACE, reducing Angiotensin II and Aldosterone

SE: Dry nagging cough, angioedema, hyperkalemia

NOTES: Can be given IV (Enalaprilat). Monitor renal function.

My Notes / Clues:

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CAPTOPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, Diabetic Nephropathy

MOA: Blocks conversion of Angiotensin I to II

SE: Cough, loss of taste, neutropenia

NOTES: Shortest half-life of ACEs. Take 1hr before meals.

My Notes / Clues:

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**BETA BLOCKER
(NON-SELECTIVE)**

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**BETA BLOCKER
(SELECTIVE)**

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**BETA BLOCKER
(SELECTIVE)**

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**BETA BLOCKER
(SELECTIVE)**

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**BETA/ALPHA
BLOCKER**

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**BETA/ALPHA
BLOCKER**

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ACE INHIBITOR

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ACE INHIBITOR

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ACE INHIBITOR

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LOSARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, diabetic nephropathy

MOA: Blocks Angiotensin II directly at the receptor site

SE: Dizziness, hyperkalemia, hypotension

NOTES: No dry cough (alternative for ACE intolerance). Teratogenic.

My Notes / Clues:

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VALSARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, Heart Failure

MOA: Blocks Angiotensin II receptors

SE: Dizziness, hyperkalemia

NOTES: Often combined with Sacubitril (Entresto) for heart failure.

My Notes / Clues:

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IRBESARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, Diabetic Nephropathy

MOA: Blocks Angiotensin II receptors

SE: Orthostatic hypotension, hyperkalemia

NOTES: Monitor BUN/Creatinine and Potassium regularly.

My Notes / Clues:

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AMLODIPINE

CLASS: Calcium Channel Blocker (DHP)

USE: HTN, chronic stable angina

MOA: Inhibits calcium influx into vascular smooth muscle

SE: Peripheral edema, dizziness, flushing

NOTES: Educate patient to report severe swelling in legs/ankles.

My Notes / Clues:

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NIFEDIPINE

CLASS: Calcium Channel Blocker (DHP)

USE: HTN, angina, preterm labor

MOA: Inhibits calcium influx into vascular smooth muscle

SE: Reflex tachycardia, peripheral edema, gingival hyperplasia

NOTES: Do not crush extended-release tablets.

My Notes / Clues:

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DILTIAZEM

CLASS: Calcium Channel Blocker (Non-DHP)

USE: AFib rate control, angina, HTN

MOA: Inhibits calcium influx, slowing AV node conduction

SE: Bradycardia, peripheral edema, constipation

NOTES: Monitor HR and BP. Used IV for rapid rate control in AFib.

My Notes / Clues:

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VERAPAMIL

CLASS: Calcium Channel Blocker (Non-DHP)

USE: Angina, SVT, HTN, migraines

MOA: Inhibits calcium influx, deeply depressing SA/AV nodes

SE: Severe constipation, bradycardia, hypotension

NOTES: Highest risk of constipation among CCBs. Increase fluids/fiber.

My Notes / Clues:

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ATORVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia, CV risk reduction

MOA: Inhibits HMG-CoA reductase, decreasing cholesterol

SE: Myopathy, rhabdomyolysis, hepatotoxicity

NOTES: Can be taken any time of day. Avoid large amounts of grapefruit juice.

My Notes / Clues:

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SIMVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia, CV event prevention

MOA: Inhibits HMG-CoA reductase

SE: Muscle pain, elevated LFTs, rhabdomyolysis

NOTES: Take in the evening. Report unexplained muscle aches.

My Notes / Clues:

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**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

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**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

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**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

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**CALCIUM CHANNEL
BLOCKER (NON-DHP)**

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**CALCIUM CHANNEL
BLOCKER (DHP)**

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**CALCIUM CHANNEL
BLOCKER (DHP)**

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**STATIN /
ANTILIPEMIC**

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**STATIN /
ANTILIPEMIC**

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**CALCIUM CHANNEL
BLOCKER (NON-DHP)**

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ROSUVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia

MOA: Inhibits HMG-CoA reductase

SE: Myopathy, elevated LFTs

NOTES: High intensity statin. Monitor CPK if muscle pain occurs.

My Notes / Clues:

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EZETIMIBE

CLASS: Cholesterol Absorption Inhibitor

USE: Hyperlipidemia

MOA: Inhibits absorption of cholesterol in the small intestine

SE: Hepatitis, myopathy, diarrhea

NOTES: Often combined with a statin for synergistic effect.

My Notes / Clues:

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CHOLESTYRAMINE

CLASS: Bile Acid Sequestrant

USE: Hyperlipidemia

MOA: Binds bile acids in the GI tract, increasing cholesterol excretion

SE: Severe constipation, bloating, belching

NOTES: Interferes with absorption of many other drugs. Space dosing out.

My Notes / Clues:

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GEMFIBROZIL

CLASS: Fibrate

USE: Hypertriglyceridemia

MOA: Decreases hepatic triglyceride secretion, increases VLDL clearance

SE: Gallstones, myopathy, hepatotoxicity

NOTES: High risk of myopathy if given concurrently with a statin.

My Notes / Clues:

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EOLOCUMAB

CLASS: PCSK9 Inhibitor

USE: Familial hypercholesterolemia, clinical ASCVD

MOA: Monoclonal antibody that increases LDL receptor recycling

SE: Injection site reactions, nasopharyngitis

NOTES: Administered via subcutaneous injection every 2-4 weeks.

My Notes / Clues:

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FUROSEMIDE

CLASS: Loop Diuretic

USE: Heart failure, severe edema, HTN

MOA: Inhibits Na/Cl reabsorption in ascending Loop of Henle

SE: Hypokalemia, ototoxicity, profound polyuria

NOTES: Push IV slowly (20mg/min) to avoid hearing loss. Monitor K+.

My Notes / Clues:

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BUMETANIDE

CLASS: Loop Diuretic

USE: Heart Failure, Edema

MOA: Inhibits Na/Cl reabsorption in Loop of Henle

SE: Profound hypokalemia, ototoxicity

NOTES: 40x more potent than furosemide. Close electrolyte monitoring required.

My Notes / Clues:

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TORSEMIDE

CLASS: Loop Diuretic

USE: Edema associated with heart failure, renal disease

MOA: Inhibits Na/Cl reabsorption in ascending Loop of Henle

SE: Hypokalemia, hypomagnesemia, dehydration

NOTES: Longer half-life and better oral bioavailability than furosemide.

My Notes / Clues:

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HYDROCHLOROTHIAZIDE

CLASS: Thiazide Diuretic

USE: HTN, peripheral edema

MOA: Inhibits Na/Cl reabsorption in distal convoluted tubule

SE: Hypokalemia, hyponatremia, hyperuricemia, hyperglycemia

NOTES: Take in the morning. Not effective if severe renal impairment.

My Notes / Clues:

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**BILE ACID
SEQUESTRANT**

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**CHOLESTEROL
ABSORPTION
INHIBITOR**

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**STATIN /
ANTILIPEMIC**

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LOOP DIURETIC

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PCSK9 INHIBITOR

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FIBRATE

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THIAZIDE DIURETIC

© 2026 Demo Study Shop

LOOP DIURETIC

© 2026 Demo Study Shop

LOOP DIURETIC

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CHLORTHALIDONE

CLASS: Thiazide-like Diuretic

USE: HTN, edema

MOA: Inhibits sodium reabsorption in the distal tubule

SE: Hypokalemia, hyperuricemia, dizziness

NOTES: Longer duration of action than HCTZ. Preferred for HTN.

My Notes / Clues:

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SPIRONOLACTONE

CLASS: Potassium-Sparing Diuretic

USE: Heart failure, ascites, HTN

MOA: Aldosterone antagonist in distal tubule

SE: Hyperkalemia, gynecomastia, amenorrhea

NOTES: Avoid potassium supplements and salt substitutes.

My Notes / Clues:

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EPLERENONE

CLASS: Potassium-Sparing Diuretic

USE: Heart failure post-MI, HTN

MOA: Selective aldosterone receptor antagonist

SE: Hyperkalemia, dizziness, hypertriglyceridemia

NOTES: Less risk of gynecomastia compared to spironolactone.

My Notes / Clues:

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ACETAZOLAMIDE

CLASS: Carbonic Anhydrase Inhibitor

USE: Glaucoma, altitude sickness, edema

MOA: Inhibits carbonic anhydrase, decreasing aqueous humor

SE: Metabolic acidosis, hypokalemia, paresthesia

NOTES: Contraindicated in patients with severe sulfa allergies.

My Notes / Clues:

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MANNITOL

CLASS: Osmotic Diuretic

USE: Increased Intracranial Pressure (ICP), Cerebral Edema

MOA: Increases osmotic pressure of glomerular filtrate

SE: Fluid/electrolyte imbalances, pulmonary edema

NOTES: Must use a filter needle/tubing. Inspect for crystals before giving.

My Notes / Clues:

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CLONIDINE

CLASS: Alpha-2 Agonist

USE: HTN, ADHD, Opioid withdrawal

MOA: Stimulates central alpha-2 receptors, decreasing sympathetic outflow

SE: Dry mouth, severe sedation, rebound hypertension

NOTES: Do not stop abruptly (hypertensive crisis risk).

My Notes / Clues:

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METHYLDOPA

CLASS: Alpha-2 Agonist

USE: Hypertension in Pregnancy

MOA: Central alpha-2 agonist, reducing sympathetic tone

SE: Sedation, positive Coombs test, hemolytic anemia

NOTES: Drug of choice for pregnancy-induced hypertension.

My Notes / Clues:

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DOXAZOSIN

CLASS: Alpha-1 Blocker

USE: HTN, BPH

MOA: Blocks peripheral alpha-1 receptors causing vasodilation

SE: First-dose orthostatic hypotension, dizziness

NOTES: Give first dose at bedtime to prevent syncope (fainting).

My Notes / Clues:

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HYDRALAZINE

CLASS: Direct Vasodilator

USE: Severe HTN, Heart Failure

MOA: Directly relaxes arteriolar smooth muscle

SE: Reflex tachycardia, fluid retention, Lupus-like syndrome

NOTES: Often given with a beta-blocker to prevent reflex tachycardia.

My Notes / Clues:

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**POTASSIUM-SPARING
DIURETIC**

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**POTASSIUM-SPARING
DIURETIC**

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**THIAZIDE-LIKE
DIURETIC**

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ALPHA-2 AGONIST

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OSMOTIC DIURETIC

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**CARBONIC
ANHYDRASE
INHIBITOR**

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**DIRECT
VASODILATOR**

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**ALPHA-1
BLOCKER**

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ALPHA-2 AGONIST

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MINOXIDIL

BLACK BOX WARNING

CLASS: Direct Vasodilator

USE: Severe, refractory HTN (also alopecia)

MOA: Opens potassium channels in smooth muscle, hyperpolarizing cell

SE: Severe fluid retention, hypertrichosis (hair growth), pericardial effusion

NOTES: Extremely potent. Only used when all other HTN drugs fail.

My Notes / Clues:

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NITROGLYCERIN

CLASS: Nitrate / Vasodilator

USE: Angina (chest pain)

MOA: Converts to nitric oxide, causing rapid venous dilation

SE: Severe headache, orthostatic hypotension, reflex tachycardia

NOTES: Keep in dark glass bottle. Call 911 if pain persists after 1 dose.

My Notes / Clues:

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ISOSORBIDE MONONITRATE

CLASS: Nitrate / Vasodilator

USE: Angina prophylaxis

MOA: Produces nitric oxide to dilate veins and coronary arteries

SE: Headache, hypotension, dizziness

NOTES: Requires a 10-14 hour nitrate-free period daily to prevent tolerance.

My Notes / Clues:

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AMIODARONE

BLACK BOX WARNING

CLASS: Class III Antiarrhythmic

USE: V-Fib, V-Tach, AFib

MOA: Prolongs action potential and refractory period (K⁺ block)

SE: Pulmonary toxicity, thyroid dysfunction, blue-gray skin, corneal deposits

NOTES: Extremely long half-life. Requires baseline PFTs, TFTs, and LFTs.

My Notes / Clues:

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SOTALOL

BLACK BOX WARNING

CLASS: Class III Antiarrhythmic

USE: Ventricular arrhythmias, AFib maintenance

MOA: Non-selective beta blocker with Class III (K⁺ block) properties

SE: Bradycardia, QT prolongation, Torsades de Pointes

NOTES: Must be initiated in a hospital with continuous ECG monitoring (3 days).

My Notes / Clues:

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FLECAINIDE

BLACK BOX WARNING

CLASS: Class IC Antiarrhythmic

USE: SVT, AFib, life-threatening ventricular arrhythmias

MOA: Potent sodium channel blockade, significantly slowing conduction

SE: Proarrhythmic (can cause lethal arrhythmias), dizziness

NOTES: Contraindicated in patients with structural heart disease or prior MI.

My Notes / Clues:

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DIGOXIN

HIGH-ALERT MEDICATION

CLASS: Cardiac Glycoside

USE: Heart Failure, Atrial Fibrillation

MOA: Inhibits Na/K ATPase pump, increasing intracellular calcium (positive inotrope)

SE: Anorexia, N/V, visual halos (yellow/green), bradycardia

NOTES: Narrow therapeutic range (0.5-2.0 ng/mL). Hypokalemia increases toxicity risk.

My Notes / Clues:

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SACUBITRIL/VALSARTAN

BLACK BOX WARNING

CLASS: ARNI

USE: Heart Failure with Reduced Ejection Fraction (HFrEF)

MOA: Neprilysin inhibitor (increases natriuretic peptides) + ARB

SE: Hypotension, hyperkalemia, angioedema

NOTES: Entresto. Must have a 36-hour washout period if switching from an ACE inhibitor.

My Notes / Clues:

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WARFARIN

BLACK BOX WARNING

CLASS: Anticoagulant (Vitamin K Antagonist)

USE: DVT, PE, AFib, Valve replacement

MOA: Inhibits synthesis of Vitamin K-dependent clotting factors (II, VII, IX, X)

SE: Bleeding, bruising, tissue necrosis

NOTES: Monitor PT/INR (Target 2-3). Maintain consistent Vit K diet. Antidote: Vitamin K.

My Notes / Clues:

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**NITRATE /
VASODILATOR**

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**NITRATE /
VASODILATOR**

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**DIRECT
VASODILATOR**

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**CLASS IC
ANTIARRHYTHMIC**

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**CLASS III
ANTIARRHYTHMIC**

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**CLASS III
ANTIARRHYTHMIC**

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**ANTICOAGULANT
(VITAMIN K
ANTAGONIST)**

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ARNI

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CARDIAC GLYCOSIDE

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HEPARIN

HIGH-ALERT MEDICATION

CLASS: Anticoagulant (Indirect Thrombin Inhibitor)

USE: Acute DVT, PE, MI, AFib

MOA: Binds to antithrombin III, inactivating thrombin and factor Xa

SE: Bleeding, Heparin-Induced Thrombocytopenia (HIT)

NOTES: Monitor aPTT. High-alert medication. Antidote: Protamine Sulfate.

My Notes / Clues:

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ENOXAPARIN

BLACK BOX WARNING

CLASS: Low Molecular Weight Heparin (LMWH)

USE: DVT prophylaxis, acute DVT/PE, ACS

MOA: Potentiates antithrombin III, preferentially inactivating factor Xa

SE: Bleeding, thrombocytopenia, injection site hematoma

NOTES: Lovenox. Give deep SQ in the abdomen. Do not expel the air bubble in the syringe.

My Notes / Clues:

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APIXABAN

BLACK BOX WARNING

CLASS: DOAC / Factor Xa Inhibitor

USE: DVT/PE treatment, AFib stroke prevention

MOA: Directly and reversibly inhibits Factor Xa

SE: Bleeding, bruising, GI hemorrhage

NOTES: Eliquis. No routine blood monitoring required. Antidote: Andexanet alfa.

My Notes / Clues:

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RIVAROXABAN

BLACK BOX WARNING

CLASS: DOAC / Factor Xa Inhibitor

USE: DVT/PE treatment, AFib

MOA: Directly inhibits Factor Xa

SE: Bleeding, bruising

NOTES: Xarelto. Take with evening meal (improves absorption). No routine INR needed.

My Notes / Clues:

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DABIGATRAN

BLACK BOX WARNING

CLASS: Direct Thrombin Inhibitor

USE: AFib stroke prevention, DVT/PE

MOA: Direct, reversible, competitive inhibition of thrombin

SE: Bleeding, dyspepsia, gastritis

NOTES: Pradaxa. Keep in original moisture-proof container. Antidote: Idarucizumab.

My Notes / Clues:

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CLOPIDOGREL

BLACK BOX WARNING

CLASS: Antiplatelet Agent

USE: Prevent MI/Stroke, post-stent placement

MOA: Irreversibly inhibits platelet aggregation (ADP receptor)

SE: Bleeding, bruising, TTP

NOTES: Plavix. Discontinue 5-7 days before planned surgery.

My Notes / Clues:

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TICAGRELOR

BLACK BOX WARNING

CLASS: Antiplatelet Agent

USE: Acute Coronary Syndrome (ACS), post-stent

MOA: Reversibly inhibits the platelet P2Y12 ADP receptor

SE: Bleeding, dyspnea (shortness of breath), bradycardia

NOTES: Brilinta. Efficacy is decreased by daily aspirin doses >100mg.

My Notes / Clues:

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ALTEPLASE (TPA)

HIGH-ALERT MEDICATION

CLASS: Thrombolytic

USE: Acute Ischemic Stroke, massive PE, STEMI

MOA: Binds to fibrin, converting plasminogen to plasmin to dissolve clots

SE: Intracranial hemorrhage, severe systemic bleeding

NOTES: Must be given within 3-4.5 hours of symptom onset for stroke. Strict exclusion criteria.

My Notes / Clues:

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**DOAC / FACTOR XA
INHIBITOR**

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**LOW MOLECULAR
WEIGHT HEPARIN
(LMWH)**

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**ANTICOAGULANT
(INDIRECT THROMBIN
INHIBITOR)**

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**ANTIPLATELET
AGENT**

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**DIRECT THROMBIN
INHIBITOR**

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**DOAC / FACTOR XA
INHIBITOR**

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THROMBOLYTIC

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**ANTIPLATELET
AGENT**

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EDUCATIONAL USE ONLY

NOT A CLINICAL REFERENCE

This study aid is a learning and exam-review tool for nursing and pharmacy students. It must not be used to make patient-care decisions, including dosing, administration, or monitoring.

ALWAYS VERIFY

Drug information, boxed (black box) warnings, and high-alert designations change over time. Before applying anything in practice, check the current FDA-approved labeling, your facility's policies, and an up-to-date drug reference. Normal lab ranges vary between laboratories; use the ranges your program or facility provides.

RECOMMENDED REFERENCES FOR VERIFICATION

- FDA drug labels: DailyMed (dailymed.nlm.nih.gov)
- ISMP List of High-Alert Medications ([ismp.org](https://www.ismp.org))
- A current nursing drug handbook or your program's required drug reference
- Your course textbook and instructor guidance

ACCURACY

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